# Panitumumab 6mg/kg Therapy

## INDICATIONS FOR USE:

| INDICATION | ICD10 | Regimen Code | HSE approved reimbursement status* |
|---|---|---|---|
| Treatment of adult patients with wild-type RAS metastatic colorectal cancer (mCRC) as monotherapy after failure of fluoropyrimidine, oxaliplatin, and irinotecan containing chemotherapy regimens | C18-20 | 00225c | N/A |

* This applies to post 2012 indications

## TREATMENT:

*The starting dose of the drugs detailed below may be adjusted downward by the prescribing clinician, using their independent medical judgement, to consider each patient's individual clinical circumstances.*

Panitumumab is administered on Day 1 of a 14-day cycle until disease progression or unacceptable toxicity.

Facilities to treat anaphylaxis MUST be present when systemic anti-cancer therapy (SACT) is administered.

| Day | Drug | Dose | Route | Diluent & Rate | Cycle |
|---|---|---|---|---|---|
| 1 | Panitumumab | 6mg/kg | IV infusion | 100mL NaCl 0.9% over 60 minutes using a 0.22 micron in-line filter[a, b, c] | Every 14 days |
| [a]Final concentration should not exceed 10mg/mL. For doses >1000mg, infuse in 150mL NaCl 0.9% over 90 minutes. | | | | | |
| [b]If the first infusion is tolerated, then subsequent infusions may be administered over 30 to 60 minutes. | | | | | |
| [c]Panitumumab is incompatible with glucose solutions.<br>Ensure IV administration sets are flushed with NaCl 0.9% pre and post administration. | | | | | |

Note: Administration volumes and fluids have been standardised to facilitate electronic prescribing system builds.

## ELIGIBILITY:

- Indication as above
- Wild type RAS tumours verified by a validated test method
- ECOG 0-2
- Adequate marrow reserve
- Adequate renal and liver function

## CAUTIONS:

- Renal impairment
- Hepatic impairment

## EXCLUSIONS:

- Hypersensitivity to panitumumab or to any of the excipients
- Patients with mutant RAS mCRC or unknown RAS mCRC status
- Patients with interstitial pneumonitis or pulmonary fibrosis



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## PRESCRIPTIVE AUTHORITY:

The treatment plan must be initiated by a Consultant Medical Oncologist.

## TESTS:

**Baseline tests**:

- FBC, liver and renal profile

**Regular tests**:

- FBC, liver and renal profile prior to each treatment
- Post treatment: monthly electrolytes, magnesium, calcium for 2 months after last panitumumab treatment

**Disease monitoring:**

Disease monitoring should be in line with the patient's treatment plan and any other test/s as directed by the supervising Consultant.

## DOSE MODIFICATIONS:

- Any dose modification should be discussed with a Consultant

### Renal and Hepatic Impairment:

**Table 1: Dose modification of panitumumab in renal and hepatic impairment**

| Renal Impairment | Hepatic Impairment | |
|---|---|---|
| | Severity | Dose |
| No need for dose adjustment is expected | Mild and moderate | No dose adjustment is needed |
| Haemodialysis: No need for dose adjustment is expected | Severe | No need for dose adjustment is expected |
| Dose modification recommendations for renal and hepatic impairment are obtained from Giraud et al. 2023. | | |

### Management of adverse events:

**Table 2: Dose modification schedule based on skin reactions**

| Occurrence of skin symptom(s): ≥ grade 3 | Administration of panitumumab | Outcome | Dose regulation |
|---|---|---|---|
| Initial occurrence | Withold 1 or 2 doses | Improved (< grade 3) | Continuing infusion at 100% of original dose |
| | | Not recovered | Discontinue |
| 2nd occurrence | Withold 1 or 2 doses | Improved (< grade 3) | Continuing infusion at 80% of original dose |
| | | Not recovered | Discontinue |
| 3rd occurrence | Withold 1 or 2 doses | Improved (< grade 3) | Continuing infusion at 60% of original dose |
| | | Not recovered | Discontinue |
| 4th occurrence | Discontinue | - | - |



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**Table 3: Dose Modification of Panitumumab for Adverse Events**

| Adverse reactions | Severity | Recommended dose modification |
|---|---|---|
| Infusion reaction | Mild or moderate | Infusion rate should be reduced for the duration of that infusion and maintain this lower infusion rate in all subsequent infusions |
| | Severe or life-threatening | Permanently discontinue |
| Interstitial lung disease | - | Permanently discontinue |
| Ulcerative keratitis | - | If a diagnosis of ulcerative keratitis is confirmed, treatment with panitumumab should be interrupted or discontinued. If keratitis is diagnosed, the benefits and risks of continuing treatment should be carefully considered. |

## SUPPORTIVE CARE:

### EMETOGENIC POTENTIAL:

- As outlined in NCCP Classification Document for Systemic Anti-Cancer Therapy (SACT) Induced Nausea and Vomiting Available on the NCCP website

**Panitumumab:** Low **(Refer to local policy).**

**For information:**
**Within NCIS regimens, antiemetics have been standardised by the Medical Oncologists and Haemato-oncologists and information is available in the following documents:**

- NCCP Supportive Care Antiemetic Medicines for **Inclusion in NCIS** (Medical Oncology) - Available on the NCCP website
- NCCP Supportive Care Antiemetic Medicines for **Inclusion in NCIS** (Haemato-oncology) - Available on the NCCP website

### PREMEDICATIONS:

Not usually required.

### OTHER SUPPORTIVE CARE:

See local skin care policy for the prevention and treatment of EGFR-inhibitor adverse skin reactions.

## ADVERSE EFFECTS:

- Please refer to the relevant Summary of Product Characteristics (SmPC) for details.

## REGIMEN SPECIFIC COMPLICATIONS:

- Please refer to the relevant Summary of Product Characteristics (SmPC) for details.
- **Infusion-related reactions**:
    - In cases of mild or moderate infusion-related reaction, the infusion rate may be decreased and maintained at the lower rate in all subsequent infusions.
    - Occurrence of a severe infusion-related reaction requires immediate and permanent discontinuation of panitumumab therapy and may necessitate emergency treatment.
    - Hypersensitivity reactions occurring more than 24 hours after infusion have been reported. Patients should be warned of the possibility of such a late onset and instructed to contact their physician if symptoms occur.
- **Respiratory disorders:**
    - Interstitial lung disease (ILD) has been observed with EGFR inhibitors. Treatment should be withheld in the event of onset or worsening respiratory symptoms. If ILD is confirmed, treatment should be discontinued.



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- **Acute renal failure:**
  - This has been observed in patients who develop severe diarrhoea and dehydration.
- **Skin reactions:**
  - This is the main adverse reaction of panitumumab. Refer to local policy for skin care regime and to Table 2 under Dose Modifications for management of treatment if patient experiences skin reactions.
- **Ocular toxicities:**
  - Patients presenting with signs and symptoms suggestive of keratitis such as acute or worsening: eye inflammation, lacrimation, light sensitivity, blurred vision, eye pain and/or red eye should be referred promptly to an ophthalmology specialist. If a diagnosis of ulcerative keratitis is confirmed, treatment should be interrupted or discontinued. If keratitis is diagnosed, the benefits and risks of continuing treatment should be carefully considered.

## DRUG INTERACTIONS:

- Current SmPC and drug interaction databases should be consulted for information

## REFERENCES:


1. Van Cutsem E, Peeters M, Siena S, et al: Open-label phase III trial of panitumumab plus best supportive care compared with best supportive care alone in patients with chemotherapy-refractory metastatic colorectal cancer. J Clin Oncol 2007;25:1658-64.
2. Amado RG**,** Wolf M, Peeters M, et al: Wild-type KRAS is required for panitumumab efficacy in patients with metastatic colorectal cancer. J Clin Oncol. 2008;26:1626-34.
3. Giraud E L, Lijster B D, et al. Dose recommendations for anticancer drugs in patients with renal or hepatic impairment: an update. Available at: https://pubmed.ncbi.nlm.nih.gov/37269847/
4. NCCP Classification Document for Systemic Anti-Cancer Therapy (SACT) Induced Nausea and Vomiting. V6 2025. Available at: https://www.hse.ie/eng/services/list/5/cancer/profinfo/chemoprotocols/nccp-classification-document-for-systemic-anti-cancer-therapy-sact-induced-nausea-and-vomiting.pdf
5. Panitumumab (VECTIBIX®) 20mg/mL Concentrate for Solution for Infusion. Summary of Product Characteristics (SmPC). Last updated 07/05/2025. Accessed 22/10/2025. Available at: https://www.ema.europa.eu/en/medicines/human/EPAR/vectibix


| Version | Date | Amendment | Approved By |
|---|---|---|---|
| 1 | 05/04/2014 | | Prof Maccon Keane |
| 2 | 30/05/2015 | Change of title to Panitumumab 6mg/kg Therapy<br>Addition of new indications, drug interactions and references | Prof Maccon Keane |
| 3 | 11/11/2016 | Amendment of indications to allow combination with FOLFIRI in first line as per SmPC | Prof Maccon Keane |
| 4 | 26/11/2018 | Updated to new NCCP template<br>Removal of combination therapy indications (a & b) - separate NCCP regimens available for these | Prof Maccon Keane |
| 5 | 10/11/2020 | Reviewed | Prof Maccon Keane |
| 6 | 21/01/2026 | Reviewed. Template and general formatting updates.Updates to ICD10 codes, exclusions, cautions, renal and hepatic impairment to Giraud et al., Table 3: dose modifications for adverse events; regimen specific complications and references section. | Prof Maccon Keane |

Comments and feedback welcome at oncologydrugs@cancercontrol.ie.



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